

CRM NO 223000502476	Name B/o. SANDHIYA	Father Name -	Lab ID 31100113460
Package BS 6	Gender MALE	DOB/Age 18/11/2023 (3 Days 19 Hrs)	Referred By Dr. D V SURESH
Hospital name SKS Hospital	City SALEM	Pre Term / Full Term Pre term	Baby Weight (Kg) 1.680
Specimen Source Venous Blood	Specimen Notes Acceptable	Collection Date & Time 22/11/2023 16:10	Received Date & Time 24/11/2023 15:00
Date of Report & time 25/11/2023 23:06	Blood Transfusion NO	Transfusion Date NO	Special Feeds / IVF / TPN/ Supplements Yes

INBORN ERRORS OF METABOLISM (IEM) SUMMARY REPORT

Sr. No.	Test Methodology	Result	Test Type	Page
1	FLUOROMETRIC ENZYME IMMUNOASSAY SCREENING REPORT	Negative	Screening	2

DISCLAIMER

All investigations have their limitations which are imposed by the limits of sensitivity and specificity of individual assay procedures as well as the quality of specimen received by the laboratory.

It is presumed that the received specimen belongs to the patient named or identified in the test request form.

Partial reproduction of this report is not permitted.

It is important to monitor baby's health and take a proactive approach should there be any unhealthy condition(s) within or beyond scope of this test.

A negative screening assay result does not rule out the possibility of an underlying metabolic / genetic disease.

The metabolites and health conditions mapping is not mutually unique and there can be other forms in which an abnormality may be apparent. Similarly, the health condition may not exclusively be an outcome of the disorders in consideration.

If the newborn is premature, retest is suggested when adjusted gestational age is 40 weeks.

HB-Variant hemoglobins may require further testing by the laboratory to diagnose.

HB-Presence of Hb Bart's with any newborn screen result indicates the presence of alpha thalassemia of unknown severity.

HB-If an increased amount of adult hemoglobin is noted, the infant is presumed to have been transfused (Including Intrauterine transfusion).

HB-Carriers of beta thalassaemia and HbLepore cannot be excluded at this age.

Investigations performed is not intended to replace the medical advice and or treatment obtained from a qualified healthcare practitioner as it doesnot cover all the metabolic disorders

HB (Covered under Hemoglobinopathies package)

Dr Anup Narayanrao Nillawar MD.,
Consultant Biochemist



CRM NO : 223000502476

NAME : B/o. SANDHIYA

REPORT- SUMMARY

Observations:

All the analytes are within the normal range.

Interpretation:

The tested sample is screened **NEGATIVE** for the list of disorders tabulated in page 2.

Comment:

It is recommended to repeat the test after 14 days in view of prematurity.

Recommendation:

Please correlate the results with other clinical and diagnostic findings.

Initial Report

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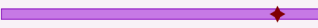
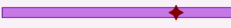
Duplicate Report

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Revision Report

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Version No

Disorder	Parameter	Result*	Result Value	Reference	Remarks
		1 10 100 1000			
Congenital Adrenal Hyperplasia	17-OHP		7.1	NMT 79 nmol/L	Normal
Glucose 6 Phosphate Dehydrogenase deficiency	G6PD		74.8	NLT 3 U/dL	Normal
Profound Biotinidase deficiency	BTD		130.7	NLT 15 U/dL	Normal
Congenital Hypothyroidism	TSH**		7.551	NMT 15 µU/mL	Normal
Phenylketonuria	PHE		75.06	NMT 168 µM/L	Normal
Classical Galactosemia	T GAL		4.4	NMT 20 mg/dL	Normal

* This Graph represents the value corresponding to results values. Each box represents the reference value. Results value is shown as dot.

* **NMT** - Not More Than, **NLT** - Not Less Than. ** TSH values are represented in Serum unit (Conversion factor: 1 µU/mL blood = 2.22 µU/mL serum).

17-OHP - 17α-OH-progesterone, G6PD - Glucose-6-Phosphate Dehydrogenase, BTD - Profound Biotinidase, TSH** - Thyroid Stimulating Hormone, PHE - Phenylalanine, T GAL - Total Galactose (galactose and galactose-1-phosphate)

REFERENCES

- <https://www.aap.org/en-us/advocacy-and-policy/aap-health-initiatives/PEHDIC/Pages/Newborn-Screening.aspx>
- Sudhanshu S, Riaz I, Sharma R, Desai MP, Parikh R, Bhatia V. Newborn Screening Guidelines for Congenital Hypothyroidism in India: Recommendations of the Indian Society for Pediatric and Adolescent Endocrinology (ISPAE) – Part II: Imaging, Treatment and Follow-up. The Indian Journal of Pediatrics. 2018 Jun 1;85(6):448–53.

** End Of Report **